

# **HOW TO OPEN A BRAIN INJURY RESIDENTIAL HOME IN COLORADO**

*An Assisted Living Residence Certified as a Supportive Living Program (SLP)  
Funded through the Colorado HCBS Brain Injury (BI) Waiver*

**A step-by-step startup and operations guide**  
Licensing • Certification • Staffing • Billing

Regulatory basis: 6 CCR 1011-1 Chapter 7 (Assisted Living Residences); Chapter 2 (General Licensure); Chapter 24 (Medication Administration); HCBS Brain Injury Waiver, 10 CCR 2505-10 § 8.7000 series (§ 8.7550).

## How to Use This Guide

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This guide walks you through opening and operating a residential home in Colorado that serves adults with brain injury under the state's Home and Community-Based Services (HCBS) Brain Injury (BI) waiver. In Colorado, that home is licensed as an Assisted Living Residence (ALR) and certified as a Supportive Living Program (SLP). There is one path: you become the licensed, certified provider yourself, enroll as a Medicaid provider agency, and bill the state directly. This guide is written for that path.

### **Important — verify before you act**

This guide is an educational resource, not legal, financial, tax, or regulatory advice.

Requirements, forms, and fees change. Confirm every step, fee, and rate directly with the Colorado Department of Public Health and Environment (CDPHE), Health Facilities and Emergency Medical Services Division, and the Department of Health Care Policy and Financing (HCPF) before you act.

The state fiscal year begins July 1, and license fees and waiver rates are updated annually. Recheck the current figures at the start of each fiscal year.

## Part 1 — What This Business Is

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A Colorado brain injury residential home is a licensed Assisted Living Residence (ALR) — a home that provides three or more unrelated adults with room and board, personal services, protective oversight, social care, and 24-hour supervision — that is also certified by the state Medicaid agency as a Supportive Living Program (SLP). The SLP certification is what allows the home to serve people on the Brain Injury waiver and to bill Medicaid for the specialized residential service.

### **What the Supported Living Program (SLP) provides**

Under 10 CCR 2505-10 § 8.7550, the SLP is a specialized, certified assisted-living service for people with brain injury. Services include:

- 24-hour oversight and protective supervision;
- Assessment, training, and supervision of self-care and independent living skills;
- Medication management;
- Behavioral management; and
- Cognitive supports.

### **Who the home serves**

The BI waiver serves individuals age 16 and older who have a brain injury of traumatic or acquired origin (occurring before the individual's 65th birthday), whose injury results in lasting physical, cognitive, emotional, or behavioral impairment of a non-progressive nature, and who meet a hospital or nursing-facility level of care (10 CCR 2505-10 §§ 8.7100, 8.7101.E). Members must be financially eligible for Health First Colorado (Medicaid) and enrolled in the BI waiver through a Case Management Agency.

## Part 2 — Who Regulates You

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Three entities govern the home, and you will work with all three:

- **CDPHE — Health Facilities and Emergency Medical Services Division.** Licenses your Assisted Living Residence and certifies your Supported Living Program. Conducts the licensing and certification surveys.
- **HCPF — Department of Health Care Policy and Financing.** Colorado's Medicaid agency. Enrolls you as a provider agency, contracts with you, and pays your claims through the BI waiver.
- **Case Management Agencies (CMAs).** Determine member eligibility and level of care, authorize services (prior authorizations), and refer members to your home. There is at least one CMA in each of the 20 defined service areas.

### The rules that apply to you

- 6 CCR 1011-1 Chapter 7 — Assisted Living Residences (your licensing and operating standard);
- 6 CCR 1011-1 Chapter 2 — General Licensure Standards;
- 6 CCR 1011-1 Chapter 24 — Medication Administration (and C.R.S. 25-1.5-301 through 303);
- 10 CCR 2505-10 § 8.7000 series — the HCBS Brain Injury waiver, including the SLP service at § 8.7550.

## Part 3 — Business and Insurance Setup

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Before you apply, put the legal and financial foundation in place:

- Form your business entity (for example, an LLC) with the Colorado Secretary of State, and keep your articles of organization, operating agreement, and other organizing documents — CDPHE requires them with your application.
- Obtain professional liability insurance held in the name of the applicant as required by the Colorado Health Care Availability Act (C.R.S. 13-64-301, et seq.), and list CDPHE as a certificate holder.
- Confirm local zoning approval for the residential use at your address, and secure the physical home (lease or purchase).
- Identify all persons and entities with a controlling interest in the home — you will disclose them on the application.

## Part 4 — The Home: Physical Plant and Fire Safety

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Your building must meet Chapter 7's physical-plant standards. For construction or renovation initiated on or after July 1, 2020, the 2018 Facilities Guidelines Institute (FGI) Guidelines for Design and Construction of Residential Health, Care, and Support Facilities apply; you submit an ALR FGI Questionnaire to request your on-site review.

### Physical plant checklist

- ☐ Sleeping rooms: at least 100 square feet for single occupancy and 60 square feet per person for double occupancy; no more than two residents per sleeping room.
- ☐ Each sleeping room has at least one window of 8 square feet with opening capability, and storage space (such as a closet) for each resident.
- ☐ At least one full bathroom (toilet, hand-washing station, mirror) for every six residents; grab bars at tubs, showers, and toilets as needed by the population.

- ☐ Accessibility for residents using auxiliary aids: accessible common/dining areas and doorways at least 32 inches wide; two means of egress if any resident uses an auxiliary aid.
- ☐ Hot water does not exceed 120°F at resident-accessible taps; safe, potable water supply.
- ☐ Oxygen stored and handled safely (secured upright, away from heat/electrical, no smoking where used); hazardous substances labeled and locked away from confused residents with a safety data sheet available.
- ☐ Fire safety: pass the local Fire Marshal inspection; disclose to residents whether the home has an automatic fire sprinkler system.

### **If you will serve residents at risk of wandering (secure environment)**

Many brain injury homes serve residents whose compromised safety awareness creates elopement risk. If you restrict free egress, you operate a “secure environment” and must meet the added requirements of Chapter 7, Part 25: enhanced disclosures and assessments (including a licensed-practitioner evaluation of wandering risk), a six-month re-assessment, secure-environment staff training, hazardous-item storage inaccessible to residents, monthly testing of securing equipment, and a secure fenced outdoor area at least six feet high. If you use egress alert devices only to alert staff and redirect residents — not to lock or restrict egress — that is not, by itself, a secure environment.

## **Part 5 — Step 1: Get Your CDPHE Assisted Living Residence License**

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The ALR license is the foundation. It is applied for through CDPHE's online health facility portal (COHFI).

- Step 1.** Submit a Letter of Intent (LOI) to CDPHE to request the Assisted Living Residence license application. Review CDPHE's “Initial Health Facility/Agency Licensing — Guidance and Instructions” and its document checklist first.
- Step 2.** Complete the application with all required attachments: your legal name and any other names used; controlling-interest persons; proof of professional liability insurance; organizing documents; the physical address (and records-storage address if different); a floor-plan map for each floor showing room layout and the services provided in each room; and your policies and procedures, in full compliance with Chapter 2 and Chapter 7.
- Step 3.** Complete the criminal history record check. An owner/applicant who has lived in Colorado more than three years requests a CBI fingerprint-based check with notification of future arrests; an owner/applicant who has lived in Colorado three years or less obtains an FBI fingerprint-based check through the CBI.
- Step 4.** Pay the non-refundable license fee. ALR fees are based on the number of licensed beds and are posted on CDPHE's Facility Fees page by March 1 each year for a July 1 effective date. Verify the current fee for your bed count.
- Step 5.** Submit the ALR FGI Questionnaire to request your on-site review, and schedule the Fire Marshal inspection.
- Step 6.** CDPHE conducts a fitness review and an on-site licensing survey. Once you pass (and clear any plan of correction), the Assisted Living Residence license is issued.

### **Where your manual and forms fit**

The application requires complete policies and procedures compliant with Chapters 2 and 7. Your Policy & Procedure Manual and the companion Forms Packet are built to satisfy this requirement and your

ongoing compliance — have them ready before you apply.

## Part 6 — Step 2: Add the SLP Certification and Enroll as a Medicaid Provider

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Once you hold (or are adding to) an ALR license, you add the Supported Living Program certification and enroll with Medicaid so you can serve BI-waiver members and get paid.

- Step 1.** Submit a Letter of Intent to CDPHE to add Supported Living Program services to your Assisted Living Residence license.
- Step 2.** CDPHE conducts an initial SLP survey. After a zero-deficiency survey, CDPHE creates a Certification & Transmittal (C&T) and uploads it to COHFI.
- Step 3.** Enroll as a Health First Colorado (Medicaid) provider agency with HCPF, attaching your C&T to the Medicaid Provider Enrollment Application. The Supported Living Program is provider Specialty 680. Obtain your National Provider Identifier (NPI). HCPF may charge a provider enrollment fee.
- Step 4.** Once enrolled and contracted, you can accept member referrals from Case Management Agencies and bill for the SLP service against each member's prior authorization.

**Note:** A provider cannot be certified for both an Alternative Care Facility (Specialty 602) and a Mental Health Transitional Living Home (755). The SLP (680) and the Brain Injury Transitional Living Program (TLP, 682) are distinct services.

## Part 7 — Staffing, Administrator, and Training

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### Administrator

Your administrator must be at least 21, hold a high school diploma or equivalent, and have at least one year of experience supervising the delivery of personal care services including activities of daily living — or meet one of the alternative qualification pathways in Chapter 7, Part 6.3 (for example, a nursing or health-care license with experience, or a relevant degree with experience). The administrator must complete 40 hours of administrator training (Parts 6.5–6.7).

### Direct-care staff

- Background checks before hire: name-based criminal history and a Colorado Adult Protective Services (CAPS) check;
- Tuberculosis testing before direct resident contact; pre-employment health screening;
- Initial orientation and role-specific training (Part 7.9), including dementia training and communication, behavioral-expression, and fall-prevention training;
- Qualified Medication Administration Persons (QMAPs) for medication, meeting Chapter 24, with a designated QMAP supervisor who verifies competency by direct observation;
- Brain-injury-specific competencies beyond the minimums: independent living skills training, behavioral management and de-escalation, cognitive supports, seizure recognition and response, and cognitive-communication strategies;
- Sufficient staffing to provide the 24-hour oversight that defines the SLP service.

## Part 8 — Billing and Getting Paid: the SLP Per-Diem

You bill the Supported Living Program as a per-diem residential service (one unit equals one day) through the Brain Injury waiver, against each member's prior authorization. The Case Management Agency completes the prior authorization; you submit claims on the CMS 1500 claim form through the Provider Web Portal using your NPI. The service is billed with Place of Service 12 (Home) under provider Specialty 680.

### SLP procedure code, acuity tiers, and per-diem rates

| Acuity Tier         | Code + Modifiers       | Per diem — Outside Denver | Per diem — Denver County |
|---------------------|------------------------|---------------------------|--------------------------|
| Tier 1              | T2033 · U6             | \$237.25                  | \$241.71                 |
| Tier 2              | T2033 · U6, HB         | \$277.39                  | \$283.96                 |
| Tier 3              | T2033 · U6, HE         | \$309.08                  | \$316.98                 |
| Tier 4              | T2033 · U6, HK         | \$370.29                  | \$380.92                 |
| Tier 5              | T2033 · U6, HB, HE     | \$407.77                  | \$420.18                 |
| Tier 6              | T2033 · U6, HB, HK     | \$453.04                  | \$467.89                 |
| Tier 7 (Negotiated) | T2033 · U6, HB, HK, SC | Facility-specific         | Facility-specific        |

**Unit:** one unit = one day (per diem). Place of Service 12 (Home); provider Specialty 680. Tier 7 is a negotiated, facility-specific rate set by the Department using the member's acuity scores. The member's acuity tier and any Denver Regional Pricing rate are authorized on the prior authorization (do not add the HX modifier for SLP).

**Rates source and date:** HCPF Home and Community Based Services — Brain Injury (BI) Waiver Rate Schedule, rates effective April 1, 2026 (Version 1.0, issued March 5, 2026). HCBS waiver rates are adjusted each state fiscal year (July 1) and can change within the year; confirm the current figures in the HCPF HCBS rate schedule before relying on them.

#### Current status — the Community First Choice (CFC) transition

The BI waiver is mid-transition to the Community First Choice (CFC) program. Between July 1, 2025 and July 1, 2026, certain attendant and support services (such as personal care, CDASS, homemaker, remote supports, and home-delivered meals) move to CFC or leave the waiver.

The residential Supported Living Program service (T2033, Specialty 680) is NOT among the services moving to CFC — it remains in the BI waiver. Still, confirm the current service menu and the current per-diem rate for each acuity tier in the HCPF BI billing manual and fee schedule before you build your financial model, and recheck at the start of each fiscal year (July 1).

## Part 9 — Timeline, Costs, and Two Worked Examples

### Rough timeline

Plan for several months from start to first billable day. The sequence is: form your entity and secure/prepare the home → submit the ALR Letter of Intent → complete the application and criminal history → FGI review, licensing survey, and Fire Marshal inspection → receive the ALR license → submit the SLP Letter of Intent → pass the SLP survey and receive your C&T → complete HCPF Medicaid provider enrollment → accept referrals and bill. Survey scheduling and any plans of correction affect the total time; verify current timeframes with CDPHE.

### Typical startup costs to budget

- The home (lease or purchase) and any FGI-compliant modifications, furnishings, and life-safety systems;
- Professional liability insurance and business setup;
- The bed-based CDPHE license fee (verify current) and any HCPF provider enrollment fee;
- Staffing and training, including administrator training and QMAP certification;
- Your Policy & Procedure Manual and Forms Packet.

### Worked example A — a 6-bed home in Colorado Springs

A single-family home is licensed as a 6-bed ALR and certified as an SLP. It serves BI-waiver members referred by the regional Case Management Agency, billing T2033 (U6 plus the tier modifiers authorized on each member's prior authorization) at the per-diem for each member's acuity tier. For illustration, using the rates above: a member authorized at Tier 3 (Outside Denver) generates \$309.08 per day; at a full census of six members near that tier, that is roughly \$1,854 per day, or about \$56,000 across a 30-day month, before expenses. Actual revenue depends on each member's authorized tier and the home's census; the home budgets 24-hour staffing to meet the SLP oversight requirement.

### Worked example B — a home in the Denver metro

A similar home operating within the City and County of Denver may qualify for Denver Regional Pricing, which raises the approved SLP per-diem — for example, Tier 3 is \$316.98 per day in Denver County versus \$309.08 outside it. That higher rate is built into the prior authorization rather than added as a claim modifier. Otherwise the licensing, certification, staffing, and billing steps are identical. Confirm current Denver Regional Pricing in the HCPF Denver Minimum Wage Regional Pricing Appendix.

## Your Compliance Foundation

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Opening the home is the first milestone; staying compliant is the ongoing work. Two companion resources are built to that end: a Policy & Procedure Manual that meets Chapter 7 and the SLP requirements and satisfies the policies-and-procedures item on your license application, and a Forms Packet containing the forms your policies reference (assessments, the medication administration record, competency tools, notices, and more). Together they give you the documentation backbone a CDPHE survey expects.

### Before you rely on any figure in this guide

Verify current fees, rates, forms, and requirements with CDPHE (Health Facilities and Emergency Medical Services Division) and HCPF.

Recheck at the start of each state fiscal year (July 1), and monitor the CFC transition for any change to the BI waiver service menu.